

Active surveillance and watchful waiting for prostate cancer

On this page

- [What is active surveillance \(monitoring\)?](#)
- [Active surveillance for early prostate cancer](#)
- [What does active surveillance involve?](#)
- [Benefits of active surveillance for early prostate cancer](#)
- [Disadvantages of active surveillance for early prostate cancer](#)
- [What is watchful waiting?](#)
- [What does watchful waiting involve](#)
- [Watchful waiting for early prostate cancer](#)
- [Watchful waiting for locally advanced prostate cancer](#)
- [How we can help](#)

What is active surveillance (monitoring)?

Active surveillance means that instead of having treatment to cure the cancer straight away, your healthcare team monitors the cancer (surveillance) with tests to check if it is growing. In low risk cancers, there is a lot of evidence that it is safe to do this.

Active surveillance for early prostate cancer

If you have [early \(localised\) prostate cancer](#) that is [low risk \(CPG1 or CPG2\)](#), your urologist may talk to you about active surveillance. They may also talk to you about active surveillance if you have [intermediate risk cancer \(CPG3\)](#) and decide you do not want surgery or radiotherapy straight away.

With active surveillance you avoid or delay side effects that can happen with [surgery](#) or [radiotherapy](#), until you need treatment. Some prostate cancers may grow so slowly that you never need treatment. Or it may be a long time before you do.

If tests show the cancer is growing more quickly or you get [symptoms](#), you will be offered treatment to cure the cancer straight away. The risk of the cancer growing without being found with tests is very low.

You will need to talk to your [urologist, cancer doctor or specialist nurse \(CNS\)](#) about whether active surveillance is right for you. They will explain the benefits and disadvantages.

You can think about how certain side effects may affect your life, as well as how you feel about how the cancer is managed. You can then decide what is the best option for you.

You can use the online decision aid called [Predict](#). You can also read more and compare treatments on [the infopool](#).

If at any time you do not feel comfortable with having active surveillance, talk to your urologist, cancer doctor or CNS.

What does active surveillance involve?

Your healthcare team will arrange for you to have regular tests. For example, this may include:

- a [multiparametric MRI scan \(mpMRI scan\)](#) when you start active surveillance – if you have not already had an MRI scan
- another mpMRI scan 12 to 18 months later
- a [PSA blood test](#) every 3 to 4 months
- a digital [rectal examination](#) after 12 months.

After the first year of active surveillance, you may have tests less often. For example, you may have:

- a PSA blood test every 6 months
- a digital rectal examination every 12 months.

Your healthcare team can tell you how often you might need tests. You may have an mpMRI scan if the cancer changes. You usually only need a [prostate biopsy](#) if there are any signs the cancer may be growing.

If the cancer is not getting any bigger or growing more quickly, it is safe to continue with active surveillance.

Benefits of active surveillance for early prostate cancer

Some possible benefits of active surveillance include the following:

- you avoid or delay having a [radical prostatectomy](#) or [radiotherapy](#).
- you can avoid or delay the side effects of treatment
- Your chance of living for another 10 years with active surveillance is the same as with radical prostatectomy or radiotherapy.

Disadvantages of active surveillance for early prostate

cancer

Some possible disadvantages of active surveillance include the following:

- You may feel very anxious about not having a treatment that could cure the cancer.
- You may find it too worrying to have to wait for test results or to look out for symptoms.
- There is a small risk the cancer may grow outside the prostate or spread to other parts of the body during active surveillance.

What is watchful waiting?

The aim of watchful waiting is to control the symptoms of [prostate cancer](#) rather than cure it. With watchful waiting, you do not have regular scans or start treatment unless the cancer is changing, or you get [symptoms](#). This means you avoid treatment and its side effects for as long as possible.

What does watchful waiting involve

You will meet with your doctor regularly. This is usually your GP. But sometimes it may be your urologist, cancer doctor or specialist nurse (CNS). They will ask if you have any new symptoms – for example, difficulty passing urine (peeing) or bone pain. If you do have symptoms, you may have regular blood tests to check your [PSA levels](#).

Many people on watchful waiting will never need any treatment for their prostate cancer. If you develop any symptoms, or your PSA level rises, your GP will refer you back to the urologist at the hospital. The urologist will usually recommend [hormonal therapy](#). This will not cure the cancer, but it can often help control it for many years.

Watchful waiting for early prostate cancer

If you have [early prostate cancer](#) your doctor may talk to you about watchful waiting if:

- the prostate cancer is not causing symptoms
- you have another medical condition that makes having treatment difficult
- you are not well enough to have a radical prostatectomy or radiotherapy
- you decide you want to avoid or delay treatment and its side effects.

Watchful waiting for locally advanced prostate cancer

If you have [locally advanced prostate cancer](#), your doctor might talk to you about watchful waiting if:

- you are older and do not have symptoms
- you have another medical condition that makes having treatment difficult
- you decide you want to avoid or delay treatment and its side effects.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our prostate cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

National Institute for Health and Care Excellence (NICE). Prostate cancer: diagnosis and management. NICE Guideline [NG131]. Published: 09 May 2019. Last updated: 15 December 2021. Available from: www.nice.org.uk/guidance/ng131 [accessed March 2024].

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Cancer Information Development team

Author

Dr Ursula McGovern

Reviewer

Consultant Medical Oncologist & Honorary Associate Professor
University College Hospitals, London

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help